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39 Psychiatric Inpatient Hospitalization Treatment Facilities (Recipients ages 21-64)

For purpose of this chapter, an inpatient is a person, age 21 through 64 years of age, who has been admitted to an Institutions for Mental Diseases (IMDs) specializing in the diagnosis, treatment, and care of serious mentally ill recipients, for the purpose of maintaining or restoring them to the greatest possible degree of health and independent functioning.

The policy provisions for IMDs can be found in the Alabama Medicaid Agency Administrative Code, Chapter 5.

39.1 Enrollment

Gainwell enrolls IMD providers and issues provider contracts to applicants who meet the licensure and/or certification requirements of the state of Alabama, the Code of Federal Regulations, the Alabama Medicaid Agency Administrative Code, and the Alabama Medicaid Provider Manual and are approved facilities under the Section 1115 Institutions for Mental Disease Waiver for Serious Mental Illness (Number 11-W-00371/4).

Refer to Chapter 2, Becoming a Medicaid Provider, for general enrollment instructions and information. Failure to provide accurate and truthful information or intentional misrepresentation might result in action ranging from denial of application to permanent exclusion.

Federal requirements mandate providers re-validate periodically with the Alabama Medicaid program. Providers will be notified when they are scheduled to re-validate. Failure to re-validate and provide appropriate documentation to complete enrollment will result in an end-date being placed on the provider file. Once a provider file has been closed for failure to timely re-validate, providers will have to submit a new application for enrollment.

National Provider Identifier, Type, and Specialty

A provider who contracts with Alabama Medicaid as a IMD is added to the Medicaid system with the National Provider Identifiers provided at the time application is made. Appropriate provider specialty codes are assigned to enable the provider to submit requests and receive reimbursements for psychiatric-related claims.

NOTE:
The 10-digit NPI is required when filing a claim.

IMDs are assigned a provider type of 01 (Hospital). The valid specialty for IMDs is (008).

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Enrollment Policy for Institutions for Mental Diseases Providers

To participate in the Alabama Medicaid Program, IMD providers must meet the following requirements:

- Receive certification for participation in the Medicaid/Medicare program
- Be accredited by the Joint Commission on Accreditation of Healthcare Organizations
- Specialize in the care and treatment of patients with serious mental illness
- Have on staff at least one full-time board-certified psychiatrist; or a full-time board certified adult psychiatrist with a minimum of 3 years' experience caring for patients 21 through 64 years of age
- Employ only staff who meet training certification standards in the area of adult psychiatry as defined by the State's mental health authority
- Provide outpatient and community liaison services throughout the state of Alabama directly or through contract with qualified providers
- Submit a written description of an acceptable utilization review plan currently in effect
- Submit a budget of cost for its inpatient services for its initial cost reporting period

  It is the facility's responsibility to ensure compliance with all federal and state regulations and to ensure that all required documentation is included in the recipient's record. Failure to comply will result in denial of payment and possible recoupment of reimbursements made previously.

Change of Ownership (CHOW) and Closures

Medicaid will mirror Medicare's Change of Ownership (CHOW) policy. Refer to Chapter 19, Hospital for additional information on Change of Ownership.

39.2 Benefits and Limitations

This section describes program-specific benefits and limitations. Refer to Chapter 3, Verifying Recipient Eligibility, for general benefit information and limitations. Refer to Chapter 7, Understanding Your Rights and Responsibilities as a Provider, for general criteria on Medical Necessity/ Medically Necessary Care.

The number of days of care charged to a recipient of inpatient psychiatric service is always a unit of a full day. A day begins at midnight and ends 24 hours later. The midnight-to-midnight method is used in reporting days of care for the recipient, even if the facility uses a different definition of day for statistical or other purposes.

Medicaid reimbursement is available for the day of admission, but not the day of discharge.

Inpatient psychiatric services for recipients ages 21 to 64, are covered services when provided under the following circumstances:

- Psychiatric services are provided in an IMD exclusively for the treatment of persons ages 21 to 64 with serious mental illness
- Psychiatric services are provided under the direction of a licensed psychiatrist
- The IMD providing services is enrolled as a Medicaid provider
- The recipient is admitted to the psychiatric facility during the entire hospitalization

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IMD services for recipients ages 21 to 64 years are unlimited if deemed medically necessary and the admission and/or the continued stay reviews meet the approved psychiatric criteria.

Certification of Need for Service

Certification of need for services is a determination that is made by a physician regarding the Medicaid recipient's treatment needs for admission to the facility.

The physician must certify for each applicant or recipient that inpatient services in a mental hospital are needed.

For non-emergency admissions, the Certificate of Need for Services Form 372 must be signed by the attending physician within 14 days of admission. For emergency admissions, Form 372 must be signed must be signed within 24 hours of admission.

For individuals applying for Medicaid while in the hospital, the certification must be made before Medicaid can authorize payment.

The physician must complete the Form #372, which is the Certification of Need for Services. This form may be obtained from the Alabama Medicaid Agency website. The following link can be used to access this form: https://medicaid.alabama.gov/documents/4.0_Programs/4.2_Medical_Services/4.2.6_Mental_Health/4.2.6.2_SMI_Waiver/4.2.6.2_SMI_Certification_Need_Services_Emergency_Admission_Psychiatric_Inpatient_Hospital_5-9-25.pdf. This form must be kept in the patient's record.

The Certificate of Needs form, which is the recertification of need for continued inpatient services, or acceptable equivalent approved by Medicaid, must be completed by a physician, a physician assistant, or a nurse practitioner acting under the supervision of a physician. The Certificate of Needs form or equivalent must be completed at least every 30 days after initial certification. This form must be kept in the patient's record.

The physician must complete an assessment note in the patient's record within 24 hours of a patient's return from any leave status.

Medical, Psychiatric, and Social Evaluation

Before authorization for payment, the attending physician, psychiatrist, or staff physician must make a medical evaluation of each individual's need for care in the facility. Appropriate professional personnel must make a psychiatric and social evaluation.

Each medical evaluation must include:

- Diagnosis
- Summary of present medical findings
- Medical history
- Mental and physical functional capacity
- Prognosis
- A recommendation by the physician concerning admission to the IMD or continued care in the psychiatric facility, for individuals who apply for Medicaid while in the facility

Plan of Care/Treatment Plan

The attending physician or staff physician must establish a written treatment plan of care for each individual at the time of admission or as soon thereafter as is reasonable and practicable before authorization of payment.

The plan of care must include the following:

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- Diagnosis, symptoms or complaints indicating a need for admission to inpatient care
- Description of the functional level of the patient
- Treatment objectives
- Orders for medications, treatments, therapies, activities, restorative/rehabilitative services, diet, social services, and special procedures needed for health and safety of the patient
- Continuing care plans that include post-discharge plans and coordination of inpatient services with partial discharge plans and related community services to ensure continuity of care with the recipient's family and community service providers upon discharge

The attending or staff physician and other appropriate staff involved in the care of the recipient must review the plan of care as needed or every 30 days.

The plan of care is evaluated to ensure that the recipient receives treatment that maintains or will restore the patient to the greatest possible level of health and independent functioning.

A written report of the evaluations and the treatment plan of care must be in the individual's record at the time of admission or immediately upon completion of the report if the individual is already in the facility.

Utilization Review (UR) Plan

As a condition of participation in the Alabama Medicaid program, each IMD must do the following:

- Have in effect a written UR Plan that provides for review of each recipient's need for services that the facility furnishes to the recipient
- Maintain recipient information required for UR, which includes the certification of need for service and the plan of care
- Provide a copy of the UR Plan and any subsequent revisions to Medicaid for review and approval

Provider Preventable Conditions (PPCs)

Provider Preventable Conditions (PPCs) are clearly defined into two separate categories: Healthcare Acquired Conditions and Other Provider Preventable Conditions (OPPCs).

Healthcare Acquired Conditions include Hospital Acquired Conditions (HACs).

OPPCs include but are not limited to the following: medication given to the wrong patient, wrong medication, wrong dose, wrong route, wrong time, wrong documentation in the medication administration records (MARs), wrong reason, and wrong response.

Non-payment of PPCs shall not prevent access to services for Medicaid beneficiaries.

To be reportable, PPCs must meet the following criteria:

- The PPC must be reasonably preventable as determined by a root cause analysis or some other means.
- The PPC must be within the control of the provider.
- The PPC must be clearly and unambiguously the result of a preventable mistake made and hospital procedures not followed, and not an event that could otherwise occur.
- The PPC must result in significant harm. The OPPCs for consideration should be limited to those that yield a serious adverse result. Serious adverse result is defined as one that results in death, a serious disability or a substantial increase

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in the duration and/or complexity of care that is well beyond the norm for treatment of the presenting condition. A serious disability is defined as a major loss of function that endures for more than 30 days, is not present at the time services were sought and is not related to the presenting condition.

- Any process for identifying non-payable events must actively incorporate some element of case-by-case review and determination. While the source and cause of some OPPCs may be clear, most would require further investigation and an internal root cause analysis to determine the cause of the serious preventable event and to assign ultimate accountability.

Pursuant to these guidelines, hospitals will not seek payments for additional days directly resulting from a PPC.

Reporting Other Provider-Preventable Conditions (OPPCs)

The following OPPCs related to the administering of medication(s) and documentation provided in the medication administration records (MARs) must be reported to Medicaid by encrypted emailing of the required information to:

adverseevents@medicaid.alabama.gov Reportable "OPPCs" include but are not limited to:

- Wrong patient
- Wrong medication
- Wrong dose
- Wrong route
- Wrong time
- Wrong documentation
- Wrong reason
- Wrong response

Reports will require the following information: Recipient first and last name, date of birth, Medicaid number, date event occurred and event type.

The IMDs may use documentation from the physician's qualifying diagnoses to identify Present of Admission which must be documented within 72 hours of the occurrence. Medicaid also recommends that the event be reported to Medicaid via e-mail within 45 days of occurrence.

Non-payment of provider preventable conditions shall not prevent access to services for Medicaid beneficiaries.

No reduction in payment for a PPC will be imposed on a hospital provider when the condition defined as a PPC for a particular patient existed prior to the initiation of treatment for that patient by that provider.

Reductions in Provider payment may be limited to the extent that the following apply:

- The Identified PPC would otherwise result in an increase in payment.
- IMDs are paid based on a daily per diem rate. It is the responsibility of the hospital to identify any Health Care-Acquired Condition and not seek payment for any additional days that have lengthened a recipient's stay due to a PPC.

  In reducing the amount of days: Hospitals are to report a value code of '81' on the UB-04 claim form along with any non-covered days and the amount field must be greater than '0'.

It is the responsibility of the IMDs to identify these events, report them, and not seek any additional payment for additional days. Medicaid will accept all POA indicators as listed below:

- Y-Yes. Diagnosis was present at time of inpatient admission
- N-No. Diagnosis was not present at time of inpatient admission

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- U-No information in the record. Documentation insufficient to determine if the condition was present at the time of inpatient admission
- W-Clinically undetermined. Provider unable to clinically determine whether the condition was present at the time of inpatient admission

It is the IMD's responsibility to include all supporting documentation with the chart for a review to be conducted by Medicaid's contracted Quality Improvement Organization (QIO). Submission of a root cause analysis is not required but may be submitted as part of the documentation to support billing.

Payment

Payment for inpatient services provided by IMDs for individuals ages 21 to 64 shall be the per diem rate established by Medicaid for the hospital. The per diem rate is based on the Medicaid cost report and all the requirements expressed in the Alabama Medicaid Administrative Code, Chapter 23.

Hospitals that terminate participation in the Medicaid program must provide a final cost report within 120 days of the date of termination of participation.

NOTE:

If a complete uniform cost report is not filed by the due date, the hospital shall be charged a penalty of $100 per day for each calendar day after the due date.

39.3 Inpatient Utilization Review

1. The determination of the level of care will be made by a licensed nurse of the hospital staff
2. All admissions will be retrospectively reviewed by the Medicaid Agency or designee as needed
3. For an individual who applies for Medicaid while in the facility, a Psychiatric Admission form must be signed by the attending physician at the time application for Medicaid is made
4. The following information shall be included on the Psychiatric Admission Form:
   (a) Recipient information:
      1. Admitting diagnosis
      2. Events leading to hospitalization
      3. History of psychiatric treatment
      4. Current medication
      5. Physician orders
      6. Presenting signs and symptoms
   (b) Events leading to present hospitalization
   (c) History and physical
   (d) Mental and physical capacity
   (e) Summary of present medical findings including prognosis and
   (f) Plan of care

39.4 Continued Stay Reviews (CSR)

The hospital's utilization review personnel are responsible for performing continued stay reviews on recipients who require continued inpatient hospitalization.

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The initial continued stay review should be performed on the date assigned by Medicaid. Later reviews should be performed at least every 30 days from the initial CSR date assigned, provided the patient is approved for continued stay. Each continued stay review date assigned should be recorded in the patient's record.

If the facility's utilization review personnel determine that the patient does not meet the criteria for continued stay, the case should be referred to the facility's psychiatric advisor. If the advisor finds that the continued stay is not needed, the hospital's utilization review procedure for denial of a continued stay should be followed.

If a final decision of denial is made, the hospital notifies the recipient and the attending physician within two days of the adverse determination. Medicaid should be notified in writing within 10 days after the denial is made.

The facility's utilization review personnel are responsible for notifying Medicaid whenever patients are placed on leave status or return from leave. A brief summary describing the outcome of the therapeutic leave should be addressed at this time for patients returning from any leave status.

39.5 Completing the Claim Form

To enhance the effectiveness and efficiency of Medicaid processing, providers should bill Medicaid claims electronically.

IMD hospital providers who bill Medicaid claims electronically receive the following benefits:

- Quicker claims processing turnaround
- Ability to immediately correct claim errors
- Online adjustments capability and
- Enhanced access to eligibility information Refer to Appendix B, Electronic Media Claims Guidelines, for more information about electronic filing

NOTE:
When an attachment is required, a hard copy UB-04 claim must be submitted.

This section describes program-specific claims information. Refer to Chapter 5, Filing Claims, for general claims filing information and instructions.

39.5.1 Time Limit for Filing Claims

Medicaid requires all claims to be filed within one year of the date of service. Refer to Section 5.1.5, Filing Limits and Approved Exceptions, for more information regarding timely filing limits and exceptions.

39.5.2 Revenue Codes

Refer to the Alabama UB-04 Manual, published by the Alabama Hospital Association, for a complete list of revenue codes.

39.5.3 Place of Service Codes

Place of service codes do not apply when filing the UB-04 claim form.

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39.6 For More Information

This section contains a cross-reference to other relevant sections in the manual.

Resource	Where to Find It
UB-04 Claim Filing Instructions	Chapter 5
Medical Necessity/Medically Necessary Care	Chapter 7
Electronic Media Claims (EMC) Guidelines	Appendix B
AVRS Quick Reference Guide	Appendix L
Alabama Medicaid Contact Information	Appendix N

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